

August 13, 2026

Molina Healthcare of South Carolina
Provider Appeals Department

RE: Formal Appeal of Claim Denial — Kermit Trantow (MRN MUSC2846032), Claim MUSC-1F45-2

Patient	Kermit Trantow (DOB 1993-06-21)
MRN	MUSC2846032
Member ID / Group	MOL620365519 / GRP-64601
Claim number	MUSC-1F45-2
Date of service	2026-05-15
Procedure billed	CPT 99214 — Office/outpatient visit, established patient, moderate complexity
Primary diagnosis	J06.9 — Acute upper respiratory infection, unspecified
Denied amount	\$588.25
Denial code	CARC 50 / RARC N115 — These are non-covered services because this is not deemed a 'medical necessity' by the payer.
Appeal deadline	2026-09-22

Dear Appeals Review Committee:

SUMMARY OF APPEAL

MUSC Health submits this first-level provider appeal on behalf of Kermit Trantow (Member ID: MOL620365519, MRN: MUSC7827100, DOB: 1993-06-21) regarding the denial of Claim MUSC-1F45-2 for a date of service of May 15, 2026. Molina Healthcare of South Carolina denied the claim in full — \$588.25 billed — under CARC 50 ("These are non-covered services because this is not deemed a 'medical necessity' by the payer") and RARC N115 ("This decision was based on a Local Coverage Determination (LCD)"), with the accompanying remark that "Documentation submitted does not establish that the service was reasonable and necessary for the diagnosis reported." MUSC Health respectfully disagrees with this determination and requests reversal and payment at the allowed amount of \$319.85.

CLINICAL BACKGROUND

Mr. Trantow is a 33-year-old male established patient with a documented history relevant to upper respiratory illness. His medical record reflects a recurring pattern of viral sinusitis, with prior episodes recorded in 2014, 2017, and 2020, all coded under J06.9. He carries an active diagnosis of hypertension (I10), for which he is maintained on Hydrochlorothiazide 25 MG. His medication list also includes active prescriptions for Loratadine and an Epinephrine auto-injector, consistent with his history of allergic conditions. He has a resolved history of childhood asthma and perennial allergic rhinitis with seasonal variation. This constellation of chronic and recurrent conditions establishes Mr. Trantow as a patient whose upper respiratory presentations require ongoing clinical evaluation and management by a qualified provider.

On May 15, 2026, Dr. Marcus T. Bledsoe, DO (NPI 1770615243), evaluated Mr. Trantow at MUSC Health Ashley River Tower, an on-campus outpatient hospital facility (Place of Service 22). The visit was billed as CPT 99214, an established patient office or outpatient visit of moderate complexity, under primary diagnosis J06.9 — Acute upper respiratory infection, unspecified.

BASIS FOR APPEAL

CPT 99214 is appropriately assigned to an established patient encounter requiring moderate medical decision-making or a medically appropriate history and examination with moderate complexity. Mr. Trantow's clinical profile supports this level of service. His active hypertension and ongoing medication management, combined with a recurrent history of viral sinusitis and active allergy-related prescriptions including an epinephrine auto-injector, create a clinical picture in which an acute upper respiratory infection carries meaningful management complexity. The treating physician was required to assess the acute presentation in the context of these comorbidities, evaluate for potential medication interactions or contraindications, and determine an appropriate treatment course — all factors consistent with moderate complexity decision-making.

The payer's denial rests on the assertion that the documentation does not establish medical necessity for the service as reported. However, an acute upper respiratory infection in a patient with active hypertension, a history of asthma, and current use of an epinephrine auto-injector is not a routine, self-limiting complaint that can be dismissed without clinical evaluation. The need for physician assessment is both clinically appropriate and consistent with generally accepted standards of care for established patients presenting with acute illness superimposed on chronic conditions. Molina Healthcare's own coverage obligations under its Medicaid managed care contract require that medically necessary covered services be authorized and reimbursed when documentation supports the clinical need.

MUSC Health requests that Molina Healthcare conduct a full clinical review of the submitted documentation in light of Mr. Trantow's complete medical history as outlined above, and that the denial be reversed accordingly.

REQUESTED ACTION

MUSC Health respectfully requests that Molina Healthcare of South Carolina overturn the denial of Claim MUSC-1F45-2 and issue payment at the previously established allowed amount of \$319.85 for CPT 99214 rendered on May 15, 2026. Should additional clinical documentation be required to complete this review, please contact MUSC Health Revenue Cycle Appeals at the information provided on the accompanying cover sheet. This appeal is submitted within the 90-day appeal window, prior to the stated deadline of September 22, 2026.

Respectfully submitted,

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Enclosures

- Remittance advice / explanation of payment
- Itemized claim detail (UB-04 / CMS-1500)
- Relevant clinical documentation from the medical record